HEALTH ECONOMICS CRASH COURSE

An Overview of the Economic Principles That Shape Healthcare Systems

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Health economics isn't just about money—it's about how we allocate limited resources to maximize health outcomes. Whether you're applying to medical school or interested in healthcare administration, understanding these principles will set you apart.

KEY TOPIC 1: SUPPLY AND DEMAND IN HEALTHCARE

The Basic Principle:

Demand = patients wanting healthcare (influenced by price, income, insurance, and health status).

Supply = providers, hospitals, and treatments available.

Why Healthcare Is Different:

Demand is often inelastic (people will pay almost anything to save a life).

Supplier-induced demand: Doctors can sometimes create demand for their own services (e.g., ordering extra tests).

Information asymmetry: Patients don't know what they need, so doctors act as agents.

KEY TOPIC 2: HEALTH INSURANCE AND MORAL HAZARD

What is Moral Hazard?

When people have insurance, they may use more healthcare than they actually need because they don't pay full price.

Example: If a doctor visits only costs a $20 copay, you might go for a minor cough instead of waiting.

How Insurance Markets Work:

Adverse selection: Sick people are more likely to buy insurance, which drives up premiums, which drives away healthy people, creating a "death spiral."

Risk pooling: Spreading risk across a large group (everyone pays in, some use it) keeps costs manageable.

Deductibles and copays are designed to reduce moral hazard by making patients pay a share.

KEY TOPIC 3: ECONOMIC EVALUATION

Types of Economic Evaluation:

1. **Cost-Effectiveness Analysis (CEA):**

   Compares the cost of an intervention to its health outcomes (often measured in QALYs or life years).

   Example: A new cancer drug costs $500,000 and extends life by 2 years. Is it worth it compared to a $50,000 drug that extends life by 1.5 years?

2. **Cost-Benefit Analysis (CBA):**

   Puts a dollar value on health outcomes (e.g., How much is a life worth?).

   Harder to do because human life is priceless, but useful for policy decisions.

3. **Cost-Utility Analysis (CUA):**

   Uses QALYs (Quality-Adjusted Life Years) as the outcome measure.

   A year of perfect health = 1 QALY; a year with severe illness = 0.2 QALYs.

> Pro Tip: When reading health policy papers, look for these terms—they're the gold standard for decision-making.

ABOUT THIS RESOURCE

This guide is part of the HealthBridge Library — free resources for every stage of your pre-health journey.

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